

The Execution Sequence

Why Order Is Not Optional — The Dependency Logic That Determines When Each Pillar Goes Live

Chapter 2 derives the six-stage execution sequence from the dependency architecture established in Chapter 1A. It uses OneCare Vermont's failure as the primary negative proof case — demonstrating with precision what breaks when the sequence is violated — and Vermont's Blueprint for Health as the primary positive proof case. It resolves the Technology-Economics dependency in detail, distinguishing Economics-as-design from Economics-as-management and Policy-as-capital from the incentive architecture. Each stage is developed with its rationale, its Vermont anchor, and its failure mode. A Vermont transformation timeline closes the chapter.

I. The Question Every Reform Architect Must Answer First

The six pillars of healthcare transformation are often presented as simultaneous imperatives. Each is necessary. Each depends on the others. Remove any one and the system fails. All of this is true. But it obscures the single most consequential practical question that every health system leader, state official, and reform architect must answer before a single dollar is spent or a single care model is redesigned: in what order do you build this?

The answer is not arbitrary. It is not the order in which the pillars are typically discussed, funded, or politically convenient. The execution sequence is determined by dependency logic — the hard reality that some things cannot function until other things are already in place. Getting the order wrong does not merely slow progress; it produces structural failures that are expensive to reverse and politically difficult to explain.

Vermont provides both the positive and negative proof cases for this argument. The Blueprint for Health — operational for nearly two decades, delivering a documented 5.8 to 1 return on investment — demonstrates what is achievable when foundational stages are built in the right order. OneCare Vermont, the state's Accountable Care Organization, demonstrates with equal clarity what happens when foundational stages are skipped or assumed. The correct execution sequence is derived from the lessons of both.

Vermont: The Crisis That Made Sequencing Non-Negotiable
The sequencing argument that follows is grounded in Vermont's fiscal reality. These numbers explain not just why Vermont needed reform — but why the sequencing of that reform could not be improvised.
Nine of Vermont's fourteen hospitals reported operating losses in FY2023, with deficits reaching -8.9%. Oliver Wyman's trajectory analysis projects thirteen of fourteen in losses by 2028 absent structural intervention. The cumulative five-year system deficit is projected between \$700 million and \$2.4 billion.
Vermont's average silver exchange plan premium rose from \$456 per month in 2018 to \$948 in 2024 — a 108% increase over six years — while median household income grew 22%. The University of Vermont Medical Center, with approximately 50% of the state's hospital market share, charged approximately 417% of Medicare rates for outpatient services in 2022. This pricing structure flows directly into premiums with no regulatory brake.
Vermont's working-age population is projected to decline 13% by 2040 — from 367,000 to approximately 318,000 — while residents aged 65+ rise from 21.7% to over 30%. This demographic shift progressively transfers patients from commercial insurance to Medicare and Medicaid, both of which reimburse hospitals below cost, collapsing the cross-subsidy that has sustained Vermont's rural hospital system.
Vermont did not choose careful sequencing because it was disciplined. It was forced to sequence correctly because it had run out of time to get it wrong. See Appendix A for the full Vermont System Portrait.

Figure 2.1 — The Six-Pillar Execution Sequence. Each stage must be operationally ready before the next assumes financial or clinical risk. Critical-path dependencies (Policy → Technology, Technology → Economics) are the two junctions where sequencing failure is most consequential. Source: HTR Analysis (2026).

II. The OneCare Failure: A Sequencing Autopsy

Before presenting the execution sequence, it is worth examining OneCare Vermont's failure in structural terms, because the failure was not random and it was not primarily the result of bad intentions or poor execution. OneCare failed because of sequencing errors baked into its design from the beginning. Identifying those errors precisely is more useful than simply noting that the ACO wound down after a decade of operation.

An Accountable Care Organization is a group of hospitals, physicians, and other providers who coordinate care for a defined patient population with the goal of reducing costs. When the ACO succeeds in keeping patients healthier and reducing unnecessary utilization, it shares in the savings generated for the payer. Vermont created OneCare as its primary ACO vehicle under the Total Cost of Care model — a payment structure that held OneCare financially responsible for the total healthcare spending of an attributed Vermont population. On paper, this was sound design. In practice, it encountered three sequencing failures that compounded into a cascade the organization could not survive.

Sequencing Failure 1 — Economics Without Policy Readiness

The most fundamental problem with OneCare was that Vermont attempted to operate a sophisticated payment reform model requiring system-wide accountability without the policy foundation to enforce participation. Provider participation in OneCare was voluntary. Hospitals, physician practices, and other providers could choose whether or not to join.

This matters structurally, not just administratively. The Total Cost of Care model holds the ACO financially accountable for the total spending of an attributed population. But when high-cost, high-revenue providers remain outside the ACO because participation is optional, the ACO is held responsible for costs it cannot influence. A hospital system generating significant revenue from inpatient admissions has a rational financial incentive to stay outside a model that rewards reducing those admissions. When it does, the ACO's ability to manage total cost is structurally undermined from day one.

What Optional Participation Actually Means for a Hospital CFO
Consider a chief financial officer of a Vermont hospital. The hospital generates \$40 million annually from inpatient admissions. OneCare Vermont is asking the hospital to join an ACO that will hold it financially accountable for reducing those admissions — reducing its revenue. Participation is voluntary.
Three choices are available: join and accept the risk; join and manage costs without affecting the revenue base; or decline to join. For many hospital systems — particularly those operating at or near breakeven in Vermont's financially stressed environment — the rational choice was the third. This is not a failure of values. It is a predictable response to an incentive structure that the policy framework failed to close.
Vermont's legislative progression from Act 167 through Act 51 to Act 68 reflects the hard-won recognition of this failure. Mandatory architecture converts voluntary reform into binding structural change. The AHEAD Model's federal agreement builds on this by embedding participation requirements that OneCare never had.

Sequencing Failure 2 — Economics Without Technology: The 'Managing Blind' Failure Mode

The second sequencing failure compounds the first. OneCare attempted to manage a complex, population-level payment model — one involving real financial risk — without the technology infrastructure required to do so. Specifically, OneCare did not build or acquire an integrated data system combining timely clinical data with claims data in a way that made population health management operationally feasible.

Claims data is the financial record of healthcare: every bill submitted to an insurer, recording what was done, where, when, and at what cost. Clinical data is the medical record: diagnoses, lab results, medications, care plans. Neither dataset is

sufficient alone for managing a value-based payment model. Both are required, integrated, and available in near real time. OneCare operated without this integrated infrastructure at meaningful scale.

The consequences were severe and entirely predictable. Care managers could not identify high-risk patients proactively — they were working from claims data arriving 60 to 90 days after care was delivered, meaning they were identifying patients who had already been hospitalized rather than preventing hospitalization in the first place.

“Managing a global budget without integrated, timely clinical and claims data is not bold reform. It is financial accountability without management capability — the conditions under which organizations can only respond to failures that have already occurred.”— *The Health Reform Portfolio*

Why Timely Data Matters as Much as Integrated Data

Consider a care manager responsible for 500 high-risk patients under a Total Cost of Care model. Her job is to identify patients who are deteriorating and intervene before they are hospitalized.

If her data arrives 60 days after the fact, she learns in March that a patient was hospitalized in January. The intervention opportunity is gone. The cost has already been incurred. She is reviewing history, not managing risk.

If her data arrives within 24 to 48 hours — integrated clinical and claims feeds updated daily — she can see that a patient filled a new prescription last week, had an abnormal lab result three days ago, and has not seen their primary care provider in six months. She can call today, before the hospitalization happens. This is the difference between retrospective reporting and proactive population health management.

Vermont's analytics vendor procurement addresses this directly: VHCURES provides longitudinal population data; VITL/VHIE provides the clinical data exchange; and the AHS-GMCB Analytics Vendor provides the modeling layer for global budget management and equity monitoring. This is the substrate OneCare never built.

Sequencing Failure 3 — The Failure Cascade

The two sequencing failures above did not operate independently. They compounded into a cascade that progressively undermined OneCare's ability to function. Because participation was optional, OneCare could not achieve the provider network coverage required to manage total cost of care. Because it could not manage total cost of care, it could not demonstrate savings. Because it could not demonstrate savings, it could not attract the capital required to build the technology infrastructure that might have allowed it to manage costs more effectively. Because it lacked that technology, its care management remained limited, its financial reporting remained inadequate, and its clinical value remained undemonstrated. And because it could not demonstrate clinical value, skeptical providers had no reason to join — returning the cascade to its starting point.

This is what a sequencing failure looks like at the system level: not a single catastrophic event, but a slow structural unraveling in which each missing foundation makes the next stage harder to execute. OneCare Vermont wound down its ACO operations after roughly a decade. The failure was architectural, not operational.

Figure 2.2 — The OneCare Vermont Failure Cascade: three sequencing errors compounding into organisational wind-down. Source: HTR Analysis (2026); GMCB records; AHS November 2025 Transformation Report.

Sequencing Gap	Structural Error	Direct Consequence	Cascade Effect
Policy (Stage 1 absent)	Voluntary participation — no mandate to require high-cost provider enrollment.	High-revenue hospitals opt out; ACO held accountable for costs it cannot influence.	Total cost of care cannot be managed at system level → savings targets missed.
Technology (Stage 2 absent)	No integrated real-time clinical + claims data; 60-90 day reporting lag.	'Managing Blind' failure mode — care managers identify hospitalised patients after the fact, not before.	Proactive intervention impossible → high-cost events not prevented → financial exposure accelerates.
Economics (Stage 3 premature)	Payment model assumed financial risk before technology was ready to support it.	Unexpected losses → inability to demonstrate savings to payers or funders.	Capital unavailable to build technology → technology gap persists → cycle repeats.
System-level cascade	Three gaps mutually reinforcing.	Organisational wind-down after approximately 10 years of operation.	Failure was architectural — no management decision could reverse it once the sequence was locked in.

Figure 2.3 — OneCare Vermont Failure Cascade: how three sequencing errors compounded into organisational wind-down. Source: HTR Analysis (2026). Based on GMCB records and public filings.

III. Resolving the Technology-Economics Dependency

The OneCare failure resolves a tension that has confused reform architects since the beginning of value-based care: the apparent chicken-and-egg relationship between Technology and Economics. The confusion runs in both directions. You cannot manage economics without technology — a global budget model without integrated, timely data is a financial accountability structure with no operational mechanism for accountability. But you cannot fund technology without economics — population health management infrastructure requires sustained capital investment, and someone must decide to fund it.

The resolution requires distinguishing three functions that reform frameworks typically conflate:

- **Economics-as-design** — the analytical work of defining the payment architecture: units of accountability, performance metrics, risk allocation, performance periods. This is planning work that can and should happen during the technology build. It does not require the technology to be running. A technology vendor building a care management platform needs to know what the payment model is measuring — design specifies the build requirements.
- **Policy-as-capital** — the funding of the technology build itself. This is a critical insight that most frameworks miss entirely: the capital for Stage 2 technology infrastructure is a Policy-pillar function, not an Economics-pillar function. Vermont's \$195 million Rural Health Transformation award is the clearest example — federal grant capital secured through the Policy pillar that funds the technology substrate. This is fundamentally different from the incentive architecture the Economics pillar creates.
- **Economics-as-management** — the ongoing operational work of running global budgets: monitoring performance in real time, calculating shared savings, adjusting payments, holding providers accountable. This work absolutely requires the technology infrastructure to already be functional. It cannot begin until the data substrate is in place.

Figure 2.4 — Resolving the Technology-Economics dependency: three distinct functions with different timing requirements. Economics-as-design and Policy-as-capital proceed in parallel with the Technology build. Economics-as-management goes live only when Technology is operational. Source: HTR Analysis (2026).

The sequencing logic that follows from this distinction is unambiguous: Policy secures both the mandate and the capital. Technology gets built — funded by Policy capital, specification driven by Economics-as-design work happening in parallel. Economics goes live as management only when the technology can support it. OneCare inverted this sequence by going live with financial risk before the technology was ready. The result was the 'Managing Blind' failure mode.

There is a second important distinction embedded in the Economics pillar that the correct sequence makes clearer: the difference between Incentive Architecture and Transformation Capital. Global budgets and reference-based pricing permanently restructure the payment environment — this is the incentive architecture that makes population health management financially rational for the long term. Transformation Capital — the RHT award, the AHEAD EAST Fund providing up to \$150 million annually — is time-limited bridge funding for infrastructure. Treating one-time grants as permanent subsidies, or treating incentive architecture as capital investment, produces reforms that are structurally misaligned from their first day of operation.

IV. The Six Stages of Execution

With OneCare's failure as the empirical anchor and the Technology-Economics dependency resolved, the six-stage execution sequence can be stated with both logical clarity and practical grounding. Each stage is developed below with its rationale, its Vermont anchor, its failure mode, and its relationship to parallel work in other stages.

Stage 1: Policy — Mandatory Architecture and Transformation Capital

Policy is the absolute precondition for everything that follows, but its role is larger than establishing regulatory permission. The Policy pillar has two functions no other pillar can perform: it creates the mandatory architecture that prevents high-cost actors from opting out of reform, and it secures the transformation capital that funds the technology build.

Vermont's legislative progression illustrates both functions. Act 167 established the diagnostic foundation — the evidence base demonstrating Vermont's healthcare system had reached a structural breaking point. Act 51 defined the transformation architecture. Act 68 operationalized the mandate — converting voluntary reform into binding statutory requirements and establishing the regulatory framework for global budgets and reference-based pricing. The AHEAD Model federal agreement layered in the federal partnership and funding commitments that made the technology build financially feasible.

The \$195 million Rural Health Transformation award is a Policy-pillar outcome, not an Economics-pillar outcome. This distinction matters because it clarifies where transformation capital comes from: not from the payment model itself, but from the policy and regulatory relationships that secure federal investment. Reform architects who expect the payment model to self-fund the technology build will consistently find themselves in the same position as OneCare — financially exposed before the infrastructure is ready.

Policy at Stage 1 must address the participation question directly. A reform architecture that allows high-cost, high-revenue providers to opt out of accountability creates a structural gap that no subsequent stage can fill. Vermont's mandatory architecture under Acts 51 and 68 is the corrective response to a decade of voluntary reform experience.

Policy Pillar — Stage 1 Checklist	Vermont Status (April 2026)
Mandatory participation authority — can high-cost actors be required to participate?	Yes — GMCB mandatory rate-setting authority; Act 68 mandatory global budgets for all 14 hospitals
Transformation capital secured — is federal/state funding in place for the technology build?	Yes — \$195M RHT Program award (Dec 2025); AHEAD State Agreement (Jan 2025)
Statutory accountability timelines — are deadlines binding, not advisory?	Yes — Act 167 monthly reporting; Act 68 statewide strategic plan by Dec 2028

Regulatory enforcement authority — does a body exist to enforce payment mandates?	Yes — GMCB; statutory authority since FY2013 hospital budget orders
Federal relationships operational — are federal agreements signed and capital flowing?	Yes — AHEAD Cohort 2 agreement live; RHT award active; EAST Fund operational Jan 2026

Figure 2.5 — Stage 1 Policy checklist with Vermont status. Source: HTR Analysis (2026).

Stage 2: Technology — Building the Substrate Before Assuming Risk

Technology is the operating system on which every subsequent stage runs. This is the sequencing insight that OneCare's failure makes most vividly clear: Technology is not a layer added on top of a functioning reform. It is the foundation that makes reform functional.

The technology infrastructure required for serious value-based transformation has three non-negotiable components. First, it must integrate clinical data with claims data. Vermont's infrastructure addresses this through VHCURES — the all-payer claims database providing foundational longitudinal data — and VTIL/VHIE, the Vermont Health Information Exchange providing the clinical data substrate. Neither alone is sufficient: VHCURES without clinical data tells you what things cost but not how to change them; clinical data without VHCURES tells you about individual patients but not population-level cost patterns.

Second, the data must be timely. Data arriving 60 to 90 days after care supports retrospective reporting but not proactive care management. Vermont's AHS-GMCB Analytics Vendor addresses this by providing the modeling layer needed for global budget management, total cost of care tracking, and equity monitoring — the analytics gap identified in the November 2025 AHS report as a critical missing piece.

Third, shared technology infrastructure must be available to rural hospitals that individually lack the scale to build these functions. Vermont's Clinically Integrated Network provides shared analytical capabilities to rural hospitals — a technology-led institution that makes population health analytics accessible to systems that cannot independently fund them.

Vermont faces its most significant current sequencing risk at this stage. The AHS-GMCB Analytics Vendor was procured in early 2026. The AHEAD Model's Medicare global budgets launch in January 2027. The analytics platform must be fully operational before that launch — not during it, not after it. Vermont is, as of this writing, executing a compressed deployment timeline that requires discipline to hold.

Stage 3: Economics — Incentive Architecture on a Visible System

Economics is the third stage, not the second, because its management function requires the technology substrate already to be operational. When the data infrastructure is in place, the Economics pillar deploys the payment architecture that converts population health management from a clinical aspiration into a financially rational imperative.

Vermont's economic architecture has two structural components that work together. Reference-based pricing, targeted at or below 200% of Medicare rates, addresses the unit price problem: Vermont's commercial insurance rates have run at 250-417% of Medicare, creating a cross-subsidy that is neither sustainable nor equitable. But price controls alone create a 'Balloon Squeeze' — if unit prices are capped without controlling volume, providers generate more volume to maintain revenue. Global hospital budgets address the volume dimension by setting a fixed total revenue envelope for each hospital, making volume expansion financially irrational. Together, these instruments break the fee-for-service trap that has made population health management economically punishing for providers.

The equity constraint on payment design becomes operationally urgent at Stage 3. Global budgets set without social risk adjustment can inadvertently penalize safety-net providers serving high-SDOH populations — providers who appear less efficient by standard metrics because their patient populations have more complex social needs. Vermont's Northeast Kingdom is the clearest example: a global budget benchmark calibrated without accounting for the region's social risk profile will generate financial pressure that reflects social disadvantage, not operational inefficiency. Social risk adjustment must be embedded in the payment design at Stage 3, not retrofitted at Stage 5.

Stage 4: Clinical — Redesigning Care on Aligned Incentives

Clinical redesign is where the economic return on the entire portfolio is realized. It is the stage at which payment incentives, technology infrastructure, and operational capacity converge to change what actually happens between a clinician and a patient. The Patient-Centered Medical Home transforms primary care from acute episodic treatment into proactive, team-based population management. The Collaborative Care Model integrates behavioral health into primary care. The Certified Community Behavioral Health Clinic expands access for the highest-need populations. Vermont's Blueprint for Health generated a documented 5.8 to 1 return on investment — among the most robust ROI findings in the value-based care literature.

Clinical redesign comes fourth because clinical transformation without economic alignment reverts when the incentives revert. This is among the most empirically robust findings from a decade of payment reform research. Clinicians redesign their practice in response to the incentives they face and the data available to them. Without a payment model that rewards the redesigned care — already live at Stage 3 — and without the technology to identify which patients need which interventions — already built at Stage 2 — clinical transformation is isolated, unsustainable, and unable to demonstrate the population-level outcomes that justify its cost.

Stage 5: Equity — Constraining Dependency and System Calibration

Equity is fifth in execution sequence but must be understood as a constraining dependency that runs through every preceding stage — not a final add-on applied to a finished system. The distinction between equity as a design constraint and equity as an audit function is critical for understanding what Stage 5 actually does.

At Stage 3, equity constrains the payment design through social risk adjustment. At Stage 4, equity constrains the clinical redesign through cultural competence requirements, language access mandates, and targeted deployment in underserved areas. Vermont's gap in primary care access between white and BIPOC residents is not closed by a Stage 5 audit — it is closed by Stages 3, 4, and 5 working in sequence: payment design that does not penalize safety-net providers, clinical redesign that deploys Community Health Teams in underserved areas, and Stage 5 audit that measures whether those interventions are actually closing the gap.

What Stage 5 adds is the systematic, population-level measurement of whether equity commitments embedded in earlier stages have produced equitable outcomes — and corrective action, informed by real data from a functioning system, when they have not. Vermont's HRSN screening at scale, the EAST Fund for social determinants, and the Rural Emergency Hospital model provide the operational mechanisms for this stage.

Stage 6: Operations — Closing the Administrative Cost Gap

Operations is the final stage and the execution layer that converts all prior-stage investments into organizational reality. Vermont's administrative cost data makes the operational opportunity concrete: Vermont's critical-access and rural hospitals spend \$2,730 per discharge on administration, compared to a national benchmark of \$1,427. This \$1,303 per-discharge gap represents the primary source of the \$100 million or more in projected direct savings from shared services and administrative simplification.

Operations is last in execution sequence not because it is least important — administrative cost reduction is one of the most direct levers for financial sustainability — but because operational redesign requires the technology infrastructure, payment incentives, and clinical models of the preceding stages to already be in place. A hospital that redesigns its administrative operations before its payment model is live will optimize for a fee-for-service world that the reform is designed to replace. The Rural Health Redesign Center methodology — performing financial and operational baselines for each of Vermont's fourteen hospitals and designing the shared services architecture — produces durable results only when the payment environment it is optimizing for is already operational.

V. The Sequence in Practice — Overlap, Iteration, and Political Reality

The six-stage sequence is a logical structure grounded in dependency relationships, not a rigid waterfall that prohibits parallel work. Payment architecture design proceeds in parallel with the technology build. Operational planning begins before the technology is complete. Clinical pilots run before the payment model fully covers the redesigned care. This is normal and expected.

What the sequence provides is a framework for understanding what happens when dependencies are violated — specifically, when a later stage assumes operational risk before an earlier stage has delivered its foundational output. When a technology build is incomplete, the payment model that depends on it should not go live with full financial risk. When a payment model has not yet aligned incentives, clinical transformation initiatives will face reversion pressure. When workforce is insufficient to deliver a clinical model, the model must be scoped to what is actually deliverable.

“The sequence failures that produce the most damage are the ones that appear to be minor timing decisions. OneCare did not dramatically choose to ignore technology. It made the reasonable-sounding decision to launch the payment model while technology was still being developed. That decision, compounded by the absence of mandatory participation, produced a failure cascade that no amount of subsequent operational excellence could reverse.” — The Health Reform Portfolio

The framework presented in this chapter gives reform architects the analytical basis to resist that pressure — to hold the sequence with discipline while managing the political reality that demands visible progress at every stage, and to distinguish between genuine parallel progress and premature sequencing that builds failure into the system from the start.

VI. Sequencing Summary

Stage	Pillar	Key Action	Why This Sequence	Vermont Anchor
1	Policy	Establish enforceable authority, mandate participation, and secure transformation capital.	Nothing can be authorized, built, or funded without statutory permission. Policy secures the capital (RHT award) that funds the technology build. Mandatory architecture prevents high-cost actors from opting out.	AHEAD Model federal agreement; Acts 167, 51, 68; \$195M RHT award; GMCB mandatory authority
2	Technology	Build integrated clinical + claims data infrastructure — real-time — before financial risk is assumed.	Technology is the operating system. It must be operational before Economics goes live. Managing global budgets without integrated data is the 'Managing Blind' failure mode that destroyed OneCare.	VHCURES all-payer claims database; VITL/VHIE clinical data exchange; AHS-GMCB Analytics Vendor; Vermont CIN; AI-assisted documentation
3	Economics	Deploy global budgets and reference-based pricing onto a system that can already see itself.	Economics-as-management requires the technology substrate to already be operational. Economics-as-design happens in parallel during Stage 2, but financial risk is not assumed until data infrastructure is ready.	Global hospital budgets; Mandatory RBP at ≤200% Medicare; AHEAD EAST Fund up to \$150M annually; TCOC model activation
4	Clinical	Redesign care delivery models and transform clinical practice on aligned incentives.	Clinical transformation is where ROI is realized — but only when payment incentives are live and technology can identify which patients need which interventions. Without aligned incentives, redesign reverts.	Blueprint for Health (5.8:1 ROI); PCMH transformation; Collaborative Care Model; CCBHC expansion (7 entities 2026)
5	Equity	Audit, calibrate, and correct — with social risk adjustment already embedded in	Equity is a constraining dependency embedded in Stages 3-4 and audited here against real population data. Without social	HRSN screening at scale; EAST Fund for SDOH; social risk adjustment in global budgets; Rural

		payment design at Stage 3.	risk adjustment, global budgets penalize safety-net providers.	Emergency Hospital model; NE Kingdom focus
6	Operations	Close the administrative cost gap and translate strategy into implemented programs.	Operations is the execution layer. Vermont's \$1,303 per-discharge administrative gap against the national benchmark is the primary source of projected direct savings from shared services and simplification.	Rural Health Redesign Center methodology; shared administrative services; 14-hospital transformation planning; Vermont CIN shared services

Figure 2.6 — The Six-Stage Execution Sequence: pillar, key action, sequencing rationale, and Vermont anchor. Source: HTR Analysis (2026).

VII. Vermont's Transformation Timeline — What Is Live When

The timeline below maps Vermont's active transformation architecture against the six-stage sequence as of April 2026. It shows what is already operational, what is in deployment, and what is planned — and where the critical sequencing risks lie. Vermont's most significant current risk is the compression between the analytics vendor deployment (Stage 2) and the AHEAD Medicare global budget launch (Stage 3) in January 2027.

Figure 2.7 — Vermont Transformation Timeline: what is live when, mapped against the six-stage sequence. Solid bars = operational or in active deployment. Lighter bars = planned/projected. Dashed red line = April 2026. Source: HTR Analysis (2026); AHS Transformation Reports (2025); GMCB records; AHEAD State Agreement.

Key Concepts Introduced in This Chapter

Term	Definition
Execution sequence	The six-stage order in which the pillars must be activated: Policy → Technology → Economics → Clinical → Equity → Operations. Derived from the dependency architecture in Chapter 1A. The sequence specifies when each pillar goes live with financial or program risk, not when planning for it begins.
Economics-as-design	The analytical work of defining the payment architecture — units of accountability, risk allocation, performance metrics — that can and should proceed in parallel with the Technology build. Does not require Technology to be operational.
Economics-as-management	The ongoing operational work of running global budgets — monitoring TCOC in real time, calculating shared savings, holding providers accountable. Requires the Technology substrate to already be fully operational. OneCare's activation of Economics-as-management without Technology is the canonical sequencing failure.
Policy-as-capital	The Policy pillar's function of securing transformation capital — federal grants, federal-state agreements, state appropriations — that funds the Technology build. Distinct from the incentive architecture the Economics pillar creates. Vermont's \$195M RHT award is Policy-as-capital.
Incentive architecture	The permanent structural features of the payment model — global budgets, reference-based pricing — that make population health management financially rational for the long term. Distinct from Transformation Capital, which is time-limited bridge funding.
Transformation capital	Time-limited bridge funding for infrastructure investment during the transformation period. Vermont examples: RHT \$195M award, AHEAD EAST Fund up to \$150M annually. Must not be confused with permanent incentive architecture.
'Balloon squeeze' risk	The failure mode in which price controls without volume controls cause providers to expand volume to maintain revenue, neutralizing the cost reduction intended by the price intervention. Global budgets address this risk by capping total revenue, not just unit prices.
Mandatory architecture	The statutory and regulatory framework that converts voluntary reform into binding obligation, preventing high-cost actors from opting out of accountability. The Policy pillar's most critical product. Vermont's Acts 51 and 68 are the mandatory architecture that OneCare's voluntary model lacked.

Key Concepts — Chapter 2. Full glossary available in the Index.

Sources

Act 167 of 2022 (Vermont); Act 51 of 2023 (Vermont); Act 68 of 2025 (Vermont); AHEAD State Agreement, Cohort 2 (January 17, 2025); Vermont Rural Health Transformation Program Application (November 2025); Oliver Wyman Healthcare and Life Sciences Group, Act 167 Community Engagement: Recommendations (August 2024, revised October 2024); Vermont Agency of Human Services Health Care System Transformation Reports (August 2025, November 2025); GMCB Hospital Budget Orders and Rate Review Records (FY2023-FY2026); OneCare Vermont Annual Reports (2017-2024); CMMI model evaluation, Congressional Budget Office (2023); Vermont Blueprint for Health Annual Report and ROI Study; Vermont Department of Health Health Equity Data Report (January 2025); AHS Hospital Transformation Plans Analysis (January 2026); HTR Analysis (2026).